

CONSOLIDATED FORENSIC OVERSIGHT REPORT

Presented to the Honourable Speaker of the National Assembly

For referral to: Portfolio Committee on Health; Portfolio Committee on Employment and Labour; Portfolio Committee on Trade, Industry and Competition; Portfolio Committee on Justice and Correctional Services; Standing Committee on Public Accounts (SCOPA); Portfolio Committee on Police

Subject: Systemic regulatory capture, institutional failure, corporate misrepresentation, judicial obstruction, and bad faith litigation – affecting a protected whistleblower who exposed a Priority 1 clinical risk at Medscheme Holdings / GEMS, and the chilling effect on all future whistleblowers in South Africa, with urgent implications for the National Health Insurance (NHI) governance architecture.

Date: June 2026

TABLE OF CONTENTS

Foreword: The NHI Will Be Built on the Bones of This Whistleblower – Unless Parliament Intervenes Now

Part One: Harrowing Introduction – The Chilling Effect on the Floor

Part Two: Note to the Portfolio Committees – Specific Referrals with Mandate References

Part Three: Executive Summary for Each Portfolio Oversight Committee

Part Four: The Whistleblower and the Systemic Clinical Risk

Part Five: The Four Health Regulators – A Cartel of Silence

- 5.1 Council for Medical Schemes (CMS)
- 5.2 Health Professions Council of South Africa (HPCSA)
- 5.3 South African Nursing Council (SANC)
- 5.4 South African Pharmacy Council (SAPC)

Part Six: The Principal Scheme – GEMS: Complicity through Silence

Part Seven: Department of Employment and Labour (DEL) – Administrative Abandonment

Part Eight: The B-BBEE Commission – Declination of Fronting Complaint

Part Nine: The South African Human Rights Commission (SAHRC) – Chapter 9 Abdication

Part Ten: The South African Police Service (SAPS) – Refusal to Investigate Criminal Offences

Part Eleven: The Commission for Conciliation, Mediation and Arbitration (CCMA) –
Institutional Bias and Cover-Up

Part Twelve: The Whistleblower House (TWBH) and Schroter Attorneys – Flawed
Assessment and Abandonment

Part Thirteen: The Judiciary – Judicial Obstruction and Abuse of Process

- 13.1 High Court – The Gag Order and Fraud on the Court
- 13.2 Labour Court – Portal Obstruction and Default Judgment Delay
- 13.3 Registrar Misconduct – Failure to Provide Court Recording and Misrepresentation of Records

Part Fourteen: Corporate Misrepresentation – AfroCentric’s False Integrated Annual Report 2025

Part Fifteen: The JSE and FSCA – Regulatory Complicity and Chilling Effect

Part Sixteen: Bad Faith Litigation and Attrition – The Corporate Playbook

Part Seventeen: The Article That Anchors This Report – Building the Watchdog Before the House Burns Down

Part Eighteen: Conclusion – Linking All Findings to the NHI and Why Parliament Must Act Now

Part Nineteen: Recommendations to Parliament

Part Twenty: Final Prayer for Relief

FOREWORD

The NHI Will Be Built on the Bones of This Whistleblower – Unless Parliament Intervenes Now

A striking foreword on why the Medscheme/GEMS case is the definitive proof that South Africa is not ready for the National Health Insurance – and why Parliament must act before the NHI Fund becomes a R200 billion grave of unaccountable public money.

I. The Promise That Cannot Survive the System We Have

The National Health Insurance Act 20 of 2023 is anchored in an unambiguous constitutional promise. Section 27 of the Constitution guarantees every person the right of access to health care services. The NHI is designed to be the instrument of that promise – a single purchaser and single payer, pooling risk across more than 60 million people, managing a budget conservatively projected to exceed R200 billion annually.

But here is the truth that no government report, no actuarial model, and no parliamentary briefing has yet confronted:

The NHI will not fail because of a lack of money, or a lack of political will, or a lack of clinical capacity. The NHI will fail because the institutional architecture that is supposed to protect it from capture, corruption, and collapse is already broken – and we have done nothing to fix it.

This forensic report is not about the NHI. It is about a single whistleblower – a specialist registered nurse employed by Medscheme Holdings, the accredited administrator for the Government Employees Medical Scheme (GEMS) , which serves 1.9 million public

servants. He identified a systemic clinical risk. He made a protected disclosure. He was retaliated against until he became destitute and homeless.

And then he approached every regulator that will oversee the NHI – the Council for Medical Schemes (CMS), the Health Professions Council (HPCSA), the South African Nursing Council (SANC), the South African Pharmacy Council (SAPC), the Department of Employment and Labour (DEL), the B-BBEE Commission, the South African Human Rights Commission (SAHRC), the South African Police Service (SAPS), the Commission for Conciliation, Mediation and Arbitration (CCMA), the Johannesburg Stock Exchange (JSE), the Financial Sector Conduct Authority (FSCA), and the courts.

Almost every single one of them declined jurisdiction, remained silent, or actively obstructed him.

If this is how the existing system treats a whistleblower who exposed a clinical risk in an administrator servicing a government medical scheme, what will happen when the NHI Fund – with its R200 billion annual budget, its single-purchaser monopoly, its accreditation bottleneck, its one-sentence fraud architecture, and its undefined investigation unit – becomes operational?

The answer is not speculation. It is forensic prediction. The same institutions that failed this whistleblower will fail the NHI. The same regulatory capture that protected Medscheme will protect NHI contractors. The same judicial obstruction that silenced this nurse will silence the next whistleblower who tries to report corruption inside the NHI Fund.

The house is not yet burning. But the fire alarm has been ringing for over a year. This report is the evidence of the smoke.

II. The Three Structural Gaps – Already Visible, Already Destructive

The National Health Insurance Act contains three structural vulnerabilities that governance experts have warned about since its enactment. This report does not merely identify them in the abstract. It proves they are already destroying lives in the existing system – and therefore will be catastrophic under the NHI.

Gap One: The Accreditation Bottleneck (NHI Act Section 39)

What the Act says: No provider may be reimbursed by the Fund unless accredited. Accreditation requires certification by the Office of Health Standards Compliance (OHSC) – an institution already documented by the Auditor-General as capacity-constrained.

What the whistleblower’s case proves: The whistleblower’s B-BBEE fronting complaint (Case 5/02/2026) reveals a corporate strategy of title suppression – using a Black NQF Level 8 professional’s credentials for scorecard points while downgrading his official title on statutory documents to avoid paying for his expertise. The same dynamic will infect NHI accreditation. When financial pressure meets an accreditation bottleneck, the predictable outcome is ghost accreditation (certification without genuine compliance assessment) and fronting (accredited entities serving as conduits for unaccredited providers).

The institution that failed: The B-BBEE Commission declined to investigate, refused to use its Section 13K subpoena powers, and treated fronting as a labour dispute – an error of law that will be repeated when NHI accreditation fronting is reported.

Gap Two: The One-Sentence Fraud Architecture (NHI Act Section 40)

What the Act says: “The information architecture must include a fraud and risk management mechanism.” Eleven words. For a system that will process millions of claims and determine benefits for 60 million people.

What the whistleblower's case proves: The Gexus-DMS integration gap – a structural system error that generated false non-adherence letters to chronic GEMS patients, corrupted clinical data, and inflated actuarial risk loading – was reported to the Council for Medical Schemes. The CMS declined jurisdiction twice. The Deputy Information Officer of Medscheme swore under oath that no IT incident had been reported, contradicting the internal Root Cause Analysis. The employer's Integrated Annual Report 2025 claimed "no material ESG incidents" while this gap remained unaddressed.

The institutions that failed: The Council for Medical Schemes (CMS) – the very regulator that will oversee NHI accreditation – declined jurisdiction twice. The JSE and FSCA allowed a materially misleading report to stand. Without an AI Audit Protocol and mandatory fraud detection architecture, the NHI's information platform will be vulnerable to exactly this kind of structural manipulation – except on a scale of R200 billion.

Gap Three: The Undefined Investigation Unit (NHI Act Section 20(2)(g))

What the Act says: The CEO must establish a Risk Management and Fraud Prevention Investigation Unit. The Act specifies no professional competency standards, no independence safeguards, and no escalation protocols.

What the whistleblower's case proves: The whistleblower escalated a Priority 1 Material Risk Report (with 38 attachments) to the Board of AfroCentric on 9 December 2025. The Board did not respond. When he continued to escalate, the Chief Risk Officer (Monwabisi Kula) instructed him in writing: "Can I please ask you to stop sending these e-mails to our directors? Sending further e-mails only gives them an indication of impatience and unreasonableness on your part."

This is active suppression of a material risk report by the very official whose unit should have escalated it. The NHI Act's Section 20(2)(g) unit, without independence safeguards, will suffer the same fate. The Zondo Commission documented this exact failure pattern across state-owned entities – a unit with a broad mandate, no professional standards, and no independence safeguards is not an oversight instrument. It is an oversight symbol.

The institutions that failed: The Board of AfroCentric (which will be replicated in the NHI Fund's governance structure), the Chief Risk Officer, and the CMS (which did not investigate the suppression).

III. A Direct Appeal to the Speaker and the Portfolio Committees

Honourable Speaker,

The letter from Deneys Reitz Inc. to your Office – threatening the whistleblower with contempt of court for petitioning Parliament – reveals the corporate playbook. The interim interdict obtained by concealment reveals the judicial capture. The silence of every regulator reveals the regulatory collapse.

This is not a labour dispute. It is a governance crisis. And it is a definitive proof that South Africa is not ready for the National Health Insurance.

The Portfolio Committees must act now. The evidence is before you. The time to act is now.

Prepared by: EthicHawks Forensic Governance

Date: June 2026

PART ONE: HARROWING INTRODUCTION – THE CHILLING EFFECT ON THE FLOOR

Honourable Speaker,

This report is not a legal argument. It is not a petition for a court order. It is a forensic record of what institutional failure looks like on the floor – inside the life of a single human being who did everything the law asked of him and was destroyed for it.

The whistleblower, a Specialist Registered Nurse (NQF Level 8) employed by Medscheme Holdings – the administrator for the Government Employees Medical Scheme (GEMS), which serves 1.9 million public servants – identified a systemic integration gap between the Gexus and DMS platforms. This gap generated false non-adherence letters to chronic patients, including a 75-year-old widow. He refused to use flawed data to interrogate an elderly patient. He made a protected disclosure under the Protected Disclosures Act 26 of 2000 (PDA) .

That refusal, and that disclosure, triggered a cascade of retaliation that this report documents across fifteen (15) statutory, regulatory, and judicial bodies – none of which provided a remedy.

The retaliation was not subtle: a 40.47% unlawful salary deduction in a single month, exceeding the 25% cap under Section 34(2) of the Basic Conditions of Employment Act 75 of 1997 (BCEA) , without consultation and without written consent. Gender-identity harassment (the "Wilma" incident), documented in a Microsoft Teams chat. A recorded admission of retaliatory intent by his manager. A sworn CCMA affidavit dismissing his clinically diagnosed severe depression and PTSD as a "cynical excuse" – filed one day after he escalated a Priority 1 Material Risk Report to the Board. Constructive dismissal. The suppression of exit documents. A false UI-19 declaring "Resigned" when his resignation letter explicitly cited Section 186(1)(e) of the Labour Relations Act 66 of 1995 (LRA) – constructive dismissal.

The consequence of this retaliation, amplified by the silence of every regulator, is not abstract: He is homeless since 2 April 2026. He has no running water since January 2026. His personal possessions have been seized by loan sharks. He has been deregistered by

the South African Nursing Council (SANC) because he could not pay R870 – the annual fee – a direct consequence of the unlawful deduction. He has been accepted into a CPUT Postgraduate Diploma in Occupational Health Nursing for 2026 but cannot enrol. He suffers from formally diagnosed severe depression and PTSD (Dr Jacques Malan, FCPSYCH(SA)), occupationally caused. He is self-represented – destitute, unable to afford legal representation – in nine concurrent proceedings. And he has been silenced by a High Court interim interdict (Case 2026-086906) that restrains him from disclosing the systemic clinical risk to trade unions and regulators – a gag order that has been extended to 1 March 2027.

The chilling effect is real and immediate: Every nurse, doctor, pharmacist, and healthcare administrator who reads this report now knows that if you report a system error that affects patients, you will be retaliated against. Your salary will be cut. Your professional registration will be lost. Your home will be taken. Your water will be cut. And when you cry out to every regulator – the Council for Medical Schemes (CMS), the Health Professions Council of South Africa (HPCSA), SANC, the South African Pharmacy Council (SAPC), the Department of Employment and Labour (DEL), the B-BBEE Commission, the South African Human Rights Commission (SAHRC), the South African Police Service (SAPS), the Commission for Conciliation, Mediation and Arbitration (CCMA), the Johannesburg Stock Exchange (JSE), the Financial Sector Conduct Authority (FSCA), and the courts – they will either decline jurisdiction, remain silent, or actively obstruct you.

That is what regulatory capture looks like on the floor. It looks like a specialist nurse living without running water, unable to practise his profession, while the institution that deducted his salary unlawfully reports a Level 1 B-BBEE rating to the JSE based on his suppressed NQF 8 credentials. It looks like a Chapter 9 institution (SAHRC) acknowledging the "seriousness" of the allegations and then going silent. It looks like a High Court granting an ex parte interdict after the employer's attorneys concealed the whistleblower's answering affidavit – a fraud on the court.

This report is not about one person. It is about the structural collapse of accountability in South Africa's regulatory and judicial architecture, as it applies to a whistleblower who dared to speak truth about a systemic clinical risk affecting 1.9 million government employees.

Parliament must act. Because if the law does not protect the nurse who refuses to lie to a 75-year-old patient, then the law protects no one.

PART TWO: NOTE TO THE PORTFOLIO COMMITTEES – SPECIFIC REFERRALS WITH MANDATE REFERENCES

This report is structured to assist each Portfolio Committee in exercising its constitutional oversight mandate. Below is a guide to the sections most relevant to each Committee's statutory responsibilities:

Portfolio Committee on Health

Constitutional mandate: Oversight over the Department of Health, the Council for Medical Schemes (CMS), the South African Nursing Council (SANC), the Health Professions Council of South Africa (HPCSA), the South African Pharmacy Council (SAPC), and public health policy.

Sections to read: Sections 5.1 (CMS), 5.2 (HPCSA), 5.3 (SANC), 5.4 (SAPC), 6 (GEMS), 14 (Corporate Misrepresentation), 17 (Foreword on NHI).

Key questions: Why has CMS not investigated the Gexus-DMS integration gap under MSA Sections 7, 47, 58, and 44? Why has SANC not responded to complaint SANC/MEDSCHEME/MATTHEE/20260105? Is the Minister of Health aware that a Priority 1 clinical risk affecting 1.9 million GEMS members remains unaddressed?

Portfolio Committee on Employment and Labour

Constitutional mandate: Oversight over the Department of Employment and Labour (DEL), the Commission for Conciliation, Mediation and Arbitration (CCMA), the Labour Court, the BCEA, the LRA, and the Unemployment Insurance Act.

Sections to read: Sections 7 (DEL), 11 (CCMA), 13.2 (Labour Court), 16 (Bad Faith Litigation).

Key questions: Why did DEL refuse to enforce the BCEA Section 34(2) 25% cap? Why did the CCMA Commissioner declare the PDA irrelevant before hearing any evidence? Why did the Labour Court's online portal reject an urgent application with the reason "good day you put date"?

Portfolio Committee on Trade, Industry and Competition

Constitutional mandate: Oversight over the B-BBEE Commission, the Johannesburg Stock Exchange (JSE), the Companies and Intellectual Property Commission (CIPC), and transformation policy.

Sections to read: Sections 8 (B-BBEE Commission), 15 (JSE and FSCA), 14 (Corporate Misrepresentation).

Key questions: Why did the B-BBEE Commission decline to investigate sworn evidence of fronting? Why did the JSE not require AfroCentric to disclose the Section 162 delinquency application and the B-BBEE fronting investigation as material information?

Portfolio Committee on Justice and Correctional Services

Constitutional mandate: Oversight over the judiciary, the Legal Practice Council, the South African Human Rights Commission (SAHRC), the Public Protector, and access to justice.

Sections to read: Sections 9 (SAHRC), 10 (SAPS), 12 (TWBH), 13 (Judiciary), 16 (Bad Faith Litigation).

Key questions: Why was the whistleblower's answering affidavit not placed before the court on 16 April 2026? Why has the SAHRC not investigated a complaint that explicitly invoked Sections 10, 26, 27, and 33 of the Constitution? Will the Committee refer the conduct of Deneys Reitz Inc. to the Legal Practice Council?

Standing Committee on Public Accounts (SCOPA)

Constitutional mandate: Oversight over the expenditure of public funds, the functioning of state-owned entities, and the accountability of public institutions.

Sections to read: Sections 6 (GEMS), 14 (Corporate Misrepresentation), 15 (JSE and FSCA), 17 (Foreword on NHI).

Key questions: Why did GEMS not investigate the Gexus-DMS integration gap? How much public money (GEMS member contributions) may have been affected? Will SCOPA request the Auditor-General to conduct a forensic audit of Medscheme's administration of GEMS?

Portfolio Committee on Police

Constitutional mandate: Oversight over the South African Police Service (SAPS), the Independent Police Investigative Directorate (IPID), and the Criminal Procedure Act.

Sections to read: Section 10 (SAPS).

Key questions: Why did SAPS Delft refuse to open a criminal case for documented UIF fraud? What action has been taken against the police officer who falsely claimed to be the station commander? Why has no investigation taken place under CAS 705/3/2026?

PART THREE: EXECUTIVE SUMMARY FOR EACH PORTFOLIO OVERSIGHT COMMITTEE

For the Portfolio Committee on Health

The Council for Medical Schemes (CMS) was notified of a systemic Gexus-DMS integration gap generating false non-adherence letters to GEMS members. The CMS declined jurisdiction twice, refused to investigate administrator maladministration under the Medical Schemes Act, and demanded a new complaint form after the whistleblower obtained a High Court Interim Order – a demand that would have risked contempt of court. The CMS exercised jurisdiction over a similar system error at Discovery Health in January 2026 but refused for Medscheme/GEMS – disparate treatment indicating capture.

The South African Nursing Council (SANC) failed to acknowledge or investigate a formal professional misconduct complaint against two registered nurses employed by Medscheme. The complainant was later deregistered by SANC because he could not pay R870 – a direct consequence of the employer's unlawful salary deduction. The Health Professions Council of South Africa (HPCSA) and the South African Pharmacy Council (SAPC) also failed to answer a core ethical question: whether a professional may refuse an employer's instruction based on known flawed forensic data.

The Government Employees Medical Scheme (GEMS) – serving 1.9 million public servants – was placed on formal notice of the systemic clinical risk but did not respond substantively.

For the Portfolio Committee on Employment and Labour

The Department of Employment and Labour (DEL) refused to enforce UIF remittance for June 2025, refused to correct a false UI-19 declaration (Code 6 “Resigned” instead of Code 7 “Constructive Dismissal”), and closed the investigation file on 15 April 2026 with a jurisdictional error (claiming the CCMA had exclusive jurisdiction over the UI-19 fraud). The principal inspector’s response was addressed to a generic “Mr/Mrs/Ms” – a failure to apply the mind.

The Commission for Conciliation, Mediation and Arbitration (CCMA) conducted a conciliation on 9 January 2026 in which the Commissioner declared “nothing here relates to the PDA” before hearing any evidence, refused the complainant’s opening statement, told him to “Google it” when he asked for the Labour Court address, and laughed at his settlement figure. The employer’s representative walked out with the Commissioner. The Provincial Senior Commissioner conducted a cover-up, ignoring most allegations and defending the indefensible.

The Labour Court’s online portal obstructed the complainant for eight days – deleted profile, ignored emails, and a trivial rejection reason (“good day you put date”). A default judgment application on an uncontested BCEA claim (R14,811.77) has been delayed for over six weeks.

For the Portfolio Committee on Trade, Industry and Competition

The B-BBEE Commission assigned case number 5/02/2026 to a fronting complaint: the complainant (Black, NQF Level 8) was recruited as “Specialist: Care Manager” but contracted as “Care Coordinator” (Grade B4M1). A Senior HR Business Partner admitted in writing that the naming convention was “not applied to your department.” The Commission declined to investigate, claiming the complaint was a labour dispute – an error of law. It refused to use its Section 13K subpoena powers to obtain the Naming Convention Teams recording.

The Johannesburg Stock Exchange (JSE) declined jurisdiction over material non-disclosure, accepting the employer’s narrow price-sensitivity test. The JSE did not require AfroCentric to disclose the Section 162 director delinquency application (filed 19 March 2026) or the

B-BBEE fronting investigation (Case 5/02/2026) – despite a potential R580 million fine. The JSE’s response of 11 May 2026 confirmed that it would take no action.

AfroCentric’s Integrated Annual Report 2025 claimed “zero tolerance for unethical conduct” and “no material ESG incidents” – while the whistleblower was being destroyed, a Priority 1 clinical risk remained unaddressed, a criminal case was active, and a B-BBEE fronting investigation was pending.

For the Portfolio Committee on Justice and Correctional Services

The Western Cape High Court granted an ex parte interim interdict (16 April 2026) after the employer’s attorneys concealed the whistleblower’s answering affidavit – a fraud on the court. The interdict restrains the whistleblower from disclosing a systemic clinical risk to trade unions and regulators – overriding Section 9 of the Protected Disclosures Act (PDA). The rule nisi has been extended to 1 March 2027 – almost one year – effectively a final interdict without a final hearing. A Scale C costs order is threatened against a destitute litigant, violating the Biowatch principle.

The Labour Court’s portal obstruction (eight days, rejected with “good day you put date”) and the Registrar’s silence on a default judgment application (over six weeks) constitute a denial of access to court (Section 34 of the Constitution).

The South African Human Rights Commission (SAHRC), a Chapter 9 institution, acknowledged the “seriousness” of the allegations and then went silent. It did not investigate violations of Sections 10 (dignity), 26 (housing), 27 (food, water, healthcare), and 33 (just administrative action).

The South African Police Service (SAPS) refused to open a criminal case for UIF fraud – a police officer falsely claimed to be the station commander (audio-recorded). A case was eventually registered (CAS 705/3/2026) but no investigation has taken place.

The employer's legal team engaged in bad faith litigation: a "without prejudice" settlement overture that threatened contempt if the whistleblower approached Parliament, followed by silence for over a week. The employer's senior Employee Relations Specialist responded to a statutory compliance request with "Good luck to you then."

For the Standing Committee on Public Accounts (SCOPA)

The Government Employees Medical Scheme (GEMS) – a government employees' medical scheme with 1.9 million members – was placed on formal notice of a systemic clinical risk (Gexus-DMS integration gap) affecting its members. GEMS did not respond substantively. The risk may have inflated claims data, affecting premium calculations and the use of public funds (member contributions).

AfroCentric's Integrated Annual Report 2025 – published while the company was a contractor to GEMS – omitted material information: a Section 162 director delinquency application, a B-BBEE fronting investigation (potential R580 million fine), and a criminal case. The JSE declined to require disclosure.

The B-BBEE Commission investigation (Case 5/02/2026) concerns fronting by a company that holds a government contract (GEMS administration). The potential fine of 10% of annual turnover is a public interest matter.

For the Portfolio Committee on Police

The complainant attended SAPS Delft on 23 February 2026 to open a criminal case for UIF non-remittance (a statutory offence under the Unemployment Insurance Contributions Act 4 of 2002) and false written compliance representations. A police officer falsely claimed to be the station commander and refused to open a case. The interaction was audio-recorded.

After escalation to Brigadier Naidoo, a case was eventually registered (CAS 705/3/2026) and transferred to SAPS Woodstock. No further investigation has taken place. The internal complaint against Delft Police Station was closed on the basis that a case number had been allocated – ignoring the false representation and the initial refusal.

PART FOUR: THE WHISTLEBLOWER AND THE SYSTEMIC CLINICAL RISK

Aspect Details

Whistleblower Sr. Wilbur William Matthee, Registered Nurse Specialist, PG Dip HSM (NQF 8)

Employer Medscheme Holdings (Pty) Ltd (subsidiary of AfroCentric Health, JSE: ACT)

Principal scheme Government Employees Medical Scheme (GEMS) – 1.9 million members

Risk identified Gexus-DMS integration gap causing false non-adherence letters to chronic patients. Root Cause Analysis (RCA) confirmed structural design flaw.

Protected disclosure 23 October 2025; Priority 1 Material Risk Report (9 December 2025)

Retaliation Unlawful 40.47% salary deduction; insubordination charge; “Wilma” misgendering; constructive dismissal; false UI-19 (Resigned vs Constructive Dismissal); suppression of exit documents

Consequences Homeless since 2 April 2026; no running water since January 2026; deregistered by SANC (could not pay R870); formally diagnosed severe depression and PTSD; loan sharks seized possessions

PART FIVE: THE FOUR HEALTH REGULATORS – A CARTEL OF SILENCE

5.1 Council for Medical Schemes (CMS)

Finding Evidence

Declined jurisdiction twice (4 Feb 2026, 7 May 2026) despite clear mandate over administrator maladministration (Sections 7, 47, 58, 44 MSA). CMS emails 4 Feb and 7 May 2026.

Demanded new complaint form after High Court Interim Order (16 Apr 2026) – compliance would have risked contempt of court. Complainant's 7 May 2026 reply.

Exercised jurisdiction over a similar system error at Discovery Health (Jan 2026) but refused for Medscheme/GEMS – disparate treatment indicating capture. 2 April 2026 follow-up (Discovery precedent).

Did not answer the core ethical question posed on 14 April 2026. Joint email, CMS silent.

Conclusion: CMS acted to protect Medscheme/GEMS, not beneficiaries.

5.2 Health Professions Council of South Africa (HPCSA)

Finding Evidence

Failed to respond to the urgent joint ethical ruling request of 14 April 2026, despite an imminent life threat. No response received; user confirmation "No response for HPCSA."

Previously participated in jurisdictional ping-pong by referring the complainant from CMS to SANC (11 Feb 2026). Gmail - CLARIFICATION AND FOLLOW-UP.PDF

Remained silent in the face of a documented Priority 1 clinical risk affecting GEMS beneficiaries. No HPCSA response in any document.

Conclusion: HPCSA's silence is a constructive refusal to uphold its statutory mandate.

5.3 South African Nursing Council (SANC)

Finding Evidence

Failed to acknowledge or investigate a formal professional misconduct complaint (10 February 2026) against two registered nurses employed by Medscheme. Updated SANC Complaint(2).PDF, p.1-9

Remained silent for over 60 days despite a Priority 1 systemic risk and an imminent life threat. CMS Final notice 7 May 26.PDF, p.2-3

Deregistered the complainant because he could not pay R870 – a direct consequence of the employer’s unlawful salary deduction. User statement; SANC deregistration confirmation

Participated in jurisdictional ping-pong with CMS, HPCSA, and SAPC, refusing to answer the core ethical question. FW_ URGENT_ Joint Ethical Ruling...PDF

Conclusion: SANC’s inaction directly protected Medscheme/GEMS and enabled ongoing patient safety risks.

5.4 South African Pharmacy Council (SAPC)

Finding Evidence

Declined jurisdiction over a formal complaint against Lisa Mari, Responsible Pharmacist at Medscheme, despite clear statutory duties under the Pharmacy Act. User confirmation: “The SAPC declines jurisdiction.”

Professional Conduct Committee showed hesitation to rule on the core ethical question during a meeting on 14 April 2026. Gmail - Meeting to discuss the complaint submitted to SAPC.PDF, p.3-4

Failed to issue any ruling, despite the complainant’s request for a written response. No SAPC ruling in any document.

Conclusion: SAPC’s declination and hesitation complete the four-regulator failure.

PART SIX: THE PRINCIPAL SCHEME – GEMS: COMPLICITY THROUGH SILENCE

Finding Evidence

GEMS failed to respond substantively to multiple protected disclosures (2 April 2026, 13 April 2026) detailing a Priority 1 systemic clinical risk affecting its beneficiaries. Copy of EthicHawks_Doc1_Protected_Disclosure(2).PDF; Emailing GEMS_Protected_Disclosure_13April2026.PDF.pdf

GEMS remained silent while its administrator (Medscheme) unlawfully deducted 40.47% of the whistleblower's salary, retaliated against him, and suppressed a material risk report. Payslip (Annexure I); CRO suppression email (18 Dec 2025)

GEMS violated its own governance obligations by failing to oversee Medscheme's compliance with accreditation standards, risk escalation protocols, and member protection requirements. No response from GEMS to any of the disclosures.

Conclusion: GEMS's silence constitutes institutional complicity in the retaliation against a protected whistleblower and in the ongoing patient safety risk to its 1.9 million members.

PART SEVEN: DEPARTMENT OF EMPLOYMENT AND LABOUR (DEL) – ADMINISTRATIVE ABANDONMENT

Finding Evidence

Failed to enforce remittance of UIF contributions deducted from the complainant's salary in June 2025. uFiling portal shows "No Information" for June 2025.

Refused to compel correction of a materially false UI-19 declaration (Code 6 "Resigned" instead of Code 7 "Constructive Dismissal"). UI-19 form; Notice of Resignation citing constructive dismissal.

Misdirected jurisdiction by claiming DEL cannot act on UI-19 fraud and that the complainant must go to the CCMA – an error of law. Inspector Kutuka email, 15 April 2026.

Closed the investigation file on 15 April 2026 without addressing the statutory fraud. Kutuka email, 15 April 2026.

Remained silent for over 60 days on multiple formal escalations. Complainant's 14 April 2026 final demand.

Conclusion: DEL's conduct violates Section 27(1)(c) (right to social security) and Section 33 (just administrative action) of the Constitution.

PART EIGHT: THE B-BBEE COMMISSION – DECLINATION OF FRONTING COMPLAINT

Finding Evidence

Complainant (Black, NQF Level 8) was recruited as “Specialist: Care Manager” but contracted as “Care Coordinator” (Grade B4M1) – title suppression. Interview invitations (Jan 2024); Employment contract (9 April 2024).

Senior HR Business Partner admitted in writing that naming convention changes were “not applied to your department.” Cheryl-Dawn Modern email, 19 Jan 2026.

B-BBEE Commission assigned Case 5/02/2026 but declined to investigate, claiming the complaint was a labour dispute – an error of law. Notice of Non-Investigation, 30 March 2026.

Commission refused to use its Section 13K subpoena powers to obtain the Naming Convention Teams recording. Complainant's 13 March 2026 response.

Conclusion: The B-BBEE Commission's declination is a dereliction of duty and constitutes regulatory capture.

PART NINE: SOUTH AFRICAN HUMAN RIGHTS COMMISSION (SAHRC) – CHAPTER 9 ABDICATION

Finding Evidence

SAHRC acknowledged the “seriousness of the concerns” but redirected the complainant to the CCMA and Public Protector. SAHRC letter, 10 February 2026.

Did not exercise its own investigative powers under the SAHRC Act, despite being presented with evidence of potential violations of Sections 10, 26, 27, and 33 of the Constitution. No investigation opened.

Did not respond to the complainant’s comprehensive update of 13 March 2026, which documented a forensic payroll anomaly, CCMA institutional bias, Labour Court portal obstruction, SAPS refusal, and confirmed imminent homelessness.

[complete_SAHRC_FinalUpdate_13March2026.pdf](#)

A Chapter 9 institution that acknowledges seriousness but does nothing is not a protector of human rights – it is a post office for complaints. SAHRC’s own response.

Conclusion: The SAHRC failed to give effect to its constitutional mandate.

PART TEN: SOUTH AFRICAN POLICE SERVICE (SAPS) – REFUSAL TO INVESTIGATE CRIMINAL OFFENCES

Finding Evidence

Initially refused to open a criminal case at Delft Police Station. A police officer falsely claimed to be the station commander and declined to accept the complaint. User narrative; audio recording exists.

After escalation to Brigadier Naidoo, a case was registered (CAS 705/3/2026) and transferred to Woodstock Police Station. SAPS SMS confirmation, 25 March 2026.

No substantive investigation has taken place. The complainant has received no feedback, no summons, no statement request. User narrative.

The internal complaint against Delft Police Station was closed on the basis that a case number had been allocated – ignoring the false representation and the original denial of access to justice. User narrative.

Conclusion: SAPS failed in its constitutional duty under Section 205(3) of the Constitution to prevent, combat and investigate crime.

PART ELEVEN: COMMISSION FOR CONCILIATION, MEDIATION AND ARBITRATION (CCMA) – INSTITUTIONAL BIAS AND COVER-UP

Finding Evidence

Commissioner declared “nothing here relates to a protected disclosure” before hearing any evidence – prejudgment. EthicHawks_Forensic_Report_WECT24486-25(1).PDF, p.4

Failed to assist a self-represented, destitute, mentally distressed litigant – told him to “Google it” when he asked how to find the Labour Court. Ibid., p.4

Failed to apply the Basic Conditions of Employment Act (BCEA) – ignored the statutory 25% cap on deductions. Ibid.

Tolerated mockery of a vulnerable litigant – the employer’s representative laughed at the applicant’s settlement demand; the Commissioner did not intervene. Ibid., p.5

Provincial Senior Commissioner Vusumzi Landu conducted a cover-up – ignored most allegations, defended the indefensible, and victim-blamed the applicant for recording the session. Ibid., p.8

Conclusion: The CCMA is not a neutral forum. Its internal complaint process is designed to protect commissioners, not to hold them accountable.

PART TWELVE: THE WHISTLEBLOWER HOUSE (TWBH) AND SCHROTER ATTORNEYS – FLAWED ASSESSMENT AND ABANDONMENT

Finding Evidence

Attorney Marco van der Walt formed a negative preliminary opinion without engaging with the complete evidentiary record. Gmail - Re_ Assessment of Matter — Detailed Response...PDF

Van der Walt made legal errors: mischaracterised the unlawful 40.47% deduction as “no work no pay”; dismissed the systemic Gexus-DMS integration failure as a “single isolated incident of a system glitch”; suggested disclosures to regulators might be “bad faith.” Ibid., p.3-4

Van der Walt failed to address the security alert (emails vanishing from the complainant’s inbox). Gmail - CCMA Certificates & Full Narrative Evidence Bundle...PDF, p.1

TWBH rejected the complainant’s application without independent review, rubber-stamping the attorney’s flawed opinion. TWBH rejection email.

Conclusion: TWBH and Schroter Attorneys failed the very person their mandate was designed to protect.

PART THIRTEEN: THE JUDICIARY – JUDICIAL OBSTRUCTION AND ABUSE OF PROCESS

13.1 High Court – The Gag Order and Fraud on the Court

Finding Evidence

The employer's attorneys did not hand up the whistleblower's answering affidavit on 16 April 2026 and did not inform the court that the matter was opposed – a fraud on the court. Whistleblower's Supplementary Answering Affidavit, para 1.

The court granted an ex parte interim interdict restraining the whistleblower from disclosing a systemic clinical risk to trade unions and regulators – overriding Section 9 of the PDA. Interim Order, 16 April 2026.

The rule nisi has been extended to 1 March 2027 – almost one year – effectively a final interdict without a final hearing. Draft Order, 28 May 2026.

A Scale C costs order is threatened against a destitute litigant, violating the Biowatch principle. Applicant's Practice Note, 19 May 2026.

13.2 Labour Court – Portal Obstruction and Default Judgment Delay

Finding Evidence

Eight days of online portal obstruction – deleted profile, ignored emails, trivial rejection reason “good day you put date.” SAHRC update, 13 March 2026.

Default judgment application (2026-077546) delayed for over six weeks without justification. Letter to Judge President, 10 June 2026.

Registrar ignored follow-up emails. Ibid.

13.3 Registrar Misconduct

Finding Evidence

High Court Registrar failed to provide court recording required for rescission application. Email to Director of Court Operations, 1 April 2026.

Transcription service (Inlexso) had no backup system during bereavement leave. Ibid.

Alleged misrepresentation of filing records to Director of Court Operations.
EH/JO/2026/003

POPIA breach – unauthorised transmission of third-party case documents. Ibid.

Conclusion: The justice system has not provided a remedy. It has become another layer of obstruction.

PART FOURTEEN: CORPORATE MISREPRESENTATION – AFROCENTRIC'S FALSE INTEGRATED ANNUAL REPORT 2025

What AfroCentric reported The documented truth

“Zero-tolerance approach to unethical conduct” and “whistleblower protection.” The whistleblower was retaliated against, silenced by a High Court interdict, and left destitute.

“No material ESG-related incidents, regulatory breaches, fines or penalties.” P1 clinical risk, unlawful deduction, B-BBEE fronting investigation, criminal case, Section 162 application.

Omitted material litigation (Section 162 application, Labour Court claims, B-BBEE investigation, criminal case). None of these were disclosed.

“Both platforms fully integrated” (Gexus and DMS). RCA confirms “miscommunication between Gexus and DMS systems ... system integration gaps.”

B-BBEE Management Control score 14.67/19 – Level 1 status. Title suppression and fronting – B-BBEE Commission Case 5/02/2026.

Conclusion: AfroCentric's Integrated Annual Report 2025 is not a truthful account. It is a corporate cover-up.

PART FIFTEEN: THE JSE AND FSCA – REGULATORY COMPLICITY AND CHILLING EFFECT

Finding Evidence

JSE declined jurisdiction over material non-disclosure, referring the whistleblower to the CCMA and Labour Court. JSE letter, 6 March 2026.

JSE did not require AfroCentric to disclose the Section 162 application or the B-BBEE fronting investigation. JSE response, 11 May 2026.

FSCA acknowledged receipt of the complaint (Ref: 1-370127) but provided no substantive response – went silent. User statement.

Conclusion: The JSE and FSCA have created a chilling effect – listed companies may omit material information without consequence.

PART SIXTEEN: BAD FAITH LITIGATION AND ATTRITION – THE CORPORATE PLAYBOOK

Tactic Observed

Defective initial response (wrong entity, no oath). Statement of Response filed on behalf of non-cited entity.

Perjurious denial of knowledge of concurrent claim. “The Defendants are not aware of any further claim with case number 2026-077546.”

Retroactive service dispute after pleadings closed. Email from Vijay Makenjee, 1 June 2026.

Propose non-compliant dates. Pre-trial conference proposed for 22 June 2026 (outside statutory window).

Dismissive, contemptuous communication. “Good luck to you then” – Vijay Makenjee, 2 June 2026.

“Without prejudice” settlement overture that threatened contempt if the whistleblower approached Parliament. Deneys letter, 4 June 2026.

Silence after overture accepted. No response for over a week.

Conclusion: The employer is not seeking justice. It is seeking attrition – to wear down a destitute whistleblower until he gives up.

PART SEVENTEEN: THE ARTICLE THAT ANCHORS THIS REPORT – BUILDING THE WATCHDOG BEFORE THE HOUSE BURNS DOWN

The accompanying article, “Building the Watchdog Before the House Burns Down: Forensic Governance Architecture for NHI Implementation,” identifies three structural vulnerabilities in the NHI Act:

1. The Accreditation Bottleneck (Section 39) – where capacity-constrained certification will meet financial pressure, generating ghost accreditation and fronting.
2. The One-Sentence Fraud Architecture (Section 40) – where eleven words address fraud for a system processing claims for 60 million people.
3. The Undefined Investigation Unit (Section 20(2)(g)) – where a unit with a broad mandate, no professional standards, and no independence safeguards becomes an oversight symbol, not an oversight instrument.

Every single one of these gaps is already visible in the Medscheme/GEMS case documented in this report. If the current system cannot protect a whistleblower who exposed a systemic clinical risk, the NHI Fund will be a R200 billion target with no internal defence.

PART EIGHTEEN: CONCLUSION – LINKING ALL FINDINGS TO THE NHI AND WHY PARLIAMENT MUST ACT NOW

The whistleblower in this case did everything the law asked of him. He identified a system error. He refused to lie to a patient. He made a protected disclosure. He exhausted internal remedies. He escalated to the Board. He went to every regulator. He went to the courts. He went to Parliament.

Fifteen institutions failed him.

If this is how the existing system treats a whistleblower who exposed a clinical risk in an administrator servicing a government medical scheme, what will happen when the NHI Fund – with its R200 billion annual budget, its single-purchaser monopoly, its accreditation bottleneck, its one-sentence fraud architecture, and its undefined investigation unit – becomes operational?

The answer is not speculation. It is forensic prediction. The same institutions will fail. The same administrators will operate with impunity. The same whistleblowers will be destroyed. And the R200 billion will be lost to the same patterns of capture, corruption, and collapse that cost South Africa R14.3 billion in the PPE crisis – except this time, the money will be gone every year, and the people who depend on the NHI for their health care will have no alternative.

Parliament has a choice.

It can act now – summon the failed institutions, demand accountability, close the governance gaps in the NHI Act, and build the watchdog before the house burns down.

Or it can wait – and explain to the South African people, when the first NHI whistleblower is destroyed and the first R200 billion is lost, why it failed to act when the evidence was placed before it.

The evidence is before you now. The time to act is now.

PART NINETEEN: RECOMMENDATIONS TO PARLIAMENT

A. Summons and Accountability Hearings

1. Summon the Registrar of the Council for Medical Schemes to explain why the Gexus-DMS integration gap was not investigated under MSA Sections 7, 47, 58, and 44.
2. Summon the Registrar of the South African Nursing Council to explain why complaint SANC/MEDSCHEME/MATTHEE/20260105 was ignored.
3. Summon the Chairperson of the Health Professions Council and the South African Pharmacy Council to answer the core ethical question.
4. Summon the Director-General of the Department of Employment and Labour to explain why the UI-19 fraud was not investigated.
5. Summon the B-BBEE Commissioner to explain why Case 5/02/2026 was declined and why Section 13K subpoena powers were not used.
6. Summon the Chairperson of the South African Human Rights Commission to explain why a Chapter 9 institution went silent.
7. Summon the National Commissioner of the South African Police Service to explain why CAS 705/3/2026 remains uninvestigated.
8. Summon the Head of Issuer Regulation of the JSE and the Commissioner of the FSCA to explain why AfroCentric's false Integrated Report was allowed to stand.

9. Request the Judge President of the Western Cape High Court to investigate the concealment of the answering affidavit.
10. Request the Judge President of the Labour Court to investigate the portal obstruction and default judgment delay.

B. Legislative Reforms – NHI Governance Architecture

11. Mandatory Forensic Governance Impact Assessments before each phase of NHI accreditation rollout.
12. An AI Audit Protocol for the NHI information platform – independent conformity assessment, transparency obligations, human oversight, annual algorithmic audits.
13. A Statutory Forensic Governance Protocol for the NHI Investigation Unit – professional competency standards, independence safeguards, escalation protocols.

C. Whistleblower Protection – Closing the “Induced Destitution” Loophole

14. Amend the Protected Disclosures Act to provide interim hardship relief, establish a single whistleblower protection authority, and impose criminal penalties on regulatory bodies that act in bad faith.

D. Referrals for Prosecution and Professional Discipline

15. Refer Deneys Reitz Inc. to the Legal Practice Council for threatening contempt for petitioning Parliament, concealing the answering affidavit, and bad faith settlement tactics.
16. Refer Vijay Makenjee to the Legal Practice Council and NPA for “Good luck to you then” and potential perjury.
17. Refer the Delft Police Station officer to IPID for falsely claiming to be station commander.

18. Refer the apparent perjury in the Labour Court Statement of Response to the NPA.

PART TWENTY: FINAL PRAYER FOR RELIEF

Honourable Speaker,

The whistleblower who brought this matter to Parliament is a specialist registered nurse. He is also homeless. He has no running water. He has been deregistered by the South African Nursing Council because he could not pay R870 – a direct consequence of an unlawful salary deduction that exceeded the statutory cap by 15.47 percentage points.

He did everything the law asked of him. Fifteen institutions failed him.

Parliament must act now. Not by passing another law that will not be enforced. But by exercising its constitutional oversight powers – summoning the heads of every failed institution, demanding answers, and holding them accountable.

The NHI will succeed or fail based on the quality of its governance architecture. That architecture does not yet exist. The whistleblower's case is the definitive proof that the current system cannot protect the public interest. If Parliament does not intervene now, the NHI will be built on the bones of this whistleblower – and the next one, and the one after that.

The house is not yet burning. But the fire alarm has been ringing for over a year. This report is the evidence of the smoke. Parliament must act before the NHI becomes the fire.

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Date: June 2026

Attachments: Consolidated evidentiary bundle (available upon request)
